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Rapid Evidence Synthesis: providing financial and debt advice in primary care

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Cite as:

Shi, C. (2023). Rapid Evidence Synthesis: providing financial and debt advice in primary care. NIHR ARC Greater Manchester: University of Manchester.

Rapid Evidence Synthesis:

Rapid Evidence Syntheses (RES) are produced by the National Institute for Health and Care Research (NIHR) Applied Research Collaboration Greater Manchester (ARC-GM). The methods used are based on a framework set out in Norman et al. 2022 and previously registered on the Open Science Framework (OSF). [1,2]

RES use evidence synthesis approaches and draw on the GRADE Evidence to Decision framework[3] to provide rapid assessments of the existing evidence and its relevance to specific decision problems. In the first instance they focus on evidence from guidance and existing evidence syntheses. They are undertaken in a real-time context of decision-making around adoption of innovative health technologies and are designed to provide a "good-enough" answer to inform decision problems in a short timescale. RES methods are flexible and adaptive. They have evolved in response to user feedback and differ depending on the nature of the assessment undertaken.

RES are not intended to serve as a substitute for a systematic review or rapid review of evidence.

We welcome feedback and are particularly interested to hear how you have used this Rapid Evidence Synthesis.

Please send any queries or comments to:

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This work was undertaken by the National Institute for Health Research (NIHR) Applied Research Collaboration Greater Manchester (ARC-GM). The views expressed are those of the author(s) and not necessarily those of the NIHR or the Department of Health and Social Care.

[1] Norman, G. *Rapid evidence synthesis to support health system decision making. OSF registration.* 2020 [cited 2023]; Available from: osf.io/hsxk5

[2] Norman, G., et al., *Rapid Evidence Synthesis To Enable Innovation And Adoption in Health and Social Care.* Systematic Reviews, 2022. **11**: p. 250. https://doi.org/10.1186/s13643-022-02106-z

[3] Alonso-Coello, P., et al., *GRADE Evidence to Decision (EtD) frameworks: a systematic and transparent approach to making well informed healthcare choices. 1: Introduction.* BMJ, 2016. **353**: p. i2016.

1. Summary

It is not very clear, due to **indirect** and **limited evidence** identified, about the additional benefits of providing financial and debt hardship advice and support in general practice and in community pharmacies. There is **consistent** and **directly relevant qualitative evidence** suggesting that Citizens Advice Bureau service recipients and/or providers fed back positive experience of using Citizens Advice Bureau services in primary care.

- There is **uncertain RCT evidence** and **indirect evidence from non-RCTs** with some methodological limitations on the impacts of providing financial and debt hardship advice and support in primary care settings. Study results appear to suggest that co-locating local Citizens Advice service staff at UK general practices for specialist welfare benefits and debts advice could improve mental health and well-being, result in financial benefits to individuals and health system, but not improve consultation rate. Mental health was improved in women and Black advice recipients.
- There is **directly relevant** UK evidence, suggesting that recipients and/or providers of Citizens Advice Bureau services in primary care **consistently** fed back positive experience in using or providing such services. Key positive aspects or perceived benefits of using Citizens Advice services in GP as reported include: confidential, non-stigmatising and familiar environment of a GP surgery; the ability to make appointments; the availability and continuity of an experienced advisor; financial gain; managed debt; beneficial social and mental health impacts; eased workload; reduced welfare-related appointments.
- There is **indirect** evidence on barriers and facilitators for delivery of and access to debt and financial hardship advice and support in primary care settings. The suggested barriers include: the complexity of social welfare system; the difficulty of billing for non-clinical services; stigmatisation; not being accessible for all advice requesters; staff and service users' lack of awareness of debt advice services; difficulties in accessing Welfare Benefits; limited support and information for health staff.

2. Methods

2.1 Description of the Intervention

Financial advice and support around hardship often includes advice on handling debt. Such advice covers a range of issues including:

- aiming to help people to reduce and manage outgoings through identifying problem areas and exploring approaches to budgeting,
- providing strategies to reduce, renegotiate or restructure payments to creditors.
- help navigating the benefits system to ensure people are correctly claiming benefits that they are entitled to.

Advice and support may be direct (i.e. community-based health professionals directly provide financial and debt advice), and it may also include referrals to other public or third sector bodies with expertise in this area. People who are experiencing financial hardship may attend primary care repeatedly with a range of mental and physical issues which may be linked to or exacerbated by insufficient money to live adequately. Provision of support and advice around financial issues within primary care, or via primary-care mediated referral to other bodies able to provide financial advice has been suggested as a possible way of addressing root causes of condition deterioration and provide one form of appropriate support.

2.2 Search

In October 2023, we searched Medline (Ovid) and the Cochrane Library (including the Cochrane Database of Systematic Reviews and the Cochrane Register of Controlled Trials). Our searches were based on the facets of financial advice or support, hardship or debt, and primary care. The search strategy is available on request. We also searched websites of NICE and NHS England and conducted citation searching.

2.3 Key Questions

Q1. What is the evidence on the impact and effectiveness of providing debt and financial hardship advice and support services for people including those of disadvantaged groups in or via primary care settings such as general practice ad community pharmacy? Particularly what is the effectiveness evidence for the services on health inequality outcomes?

Q2. What evidence is available on people's experience of debt and financial hardship advice and support services in or via primary care settings such as general practice and pharmacy?

Q3. What factors are reported as barriers and facilitators for delivery of and access to debt and financial hardship advice and support services in people particularly those of disadvantaged groups?

2.4 Inclusion Criteria

2.4.1 Participants

We included evidence on community-dwelling adults who used primary care services including general practice and community pharmacies. We also included evidence relating specifically to disadvantaged groups.

2.4.2 Intervention

We included any type of service providing advice, guidance or support that directly focused on aspects of financial hardship, including debt. Services should be delivered in (or aimed at) a primary care or community setting such as general practice or a pharmacy. Services delivered in secondary care settings may be considered in the absence of directly relevant evidence.

Various types of debt and financial hardship advice and guidance services are available for different populations. For this RES, we focused on the services for general populations but also included evidence relating services designed for specific groups such as parents, women, older or younger age groups, people from minority ethnic groups, people with disabilities. We accepted study authors' definitions of debt and financial hardship advice, support and guidance services.

2.4.3 Comparator

We considered any comparator groups that included no intervention, care or service as usual, and alternative interventions.

2.4.4 Outcomes

For Q1 we accepted any outcome which reflects impact or effectiveness of the intervention for either participants or services. Outcomes could relate to use of healthcare services, such as number of repeat attendances in primary care, or to measures of participant health or satisfaction with services. We also accepted outcomes related to uptake or completion of programmes. For Q2 we considered participants' experience of using the intervention. For Q3 we considered identified barriers and facilitators to delivery of and use of financial hardship services.

2.4.5 Study design

In the first instance we considered existing evidence syntheses, including systematic and scoping reviews, of primary studies of any design. We considered quantitative, qualitative and mixed methods reviews as appropriate to the question addressed. We used a broad definition of systematic reviews as having a systematic search and clear inclusion criteria.

Where we were unable to identify relevant evidence syntheses, we considered primary studies, looking at the most robust primary study designs first. For Q1 these are randomised controlled trials (RCTs). For Q2 and Q3 these are well conducted qualitative or mixed methods studies. Studies with less rigorous approaches were considered in the absence of more useful evidence.

3. Results

3.1 Results of search

We identified 3717 records from database searches. After study selection, four primary studies were considered eligible for inclusion in this RES.(1-4) Searching additional sources identified one scoping review (5) and further three primary studies.(6-8) In total this RES included one scoping review and seven primary studies.

3.2 The impact of providing debt and financial advice and services (Q1)

We were unable to identify any relevant evidence synthesis for this question. We identified two pilot RCTs (2, 3) and a quasi-experimental study (4) which were relevant to Q1, all comparing debt or financial advice provision with no provision. We also included a historically controlled cohort study that included a prospective cohort who received the GP-based financial advice in comparison with retrospective cohort of populations receiving health centres-financial advice (that served broader localities than GP-based financial advice).(7)

Gabbay (2017) reports pilot phase results of a UK-based RCT that aimed to assess the clinical and cost-effectiveness of adding a primary care debt counselling advice service to usual care for patients with depression and debt.(3) The trial collected data for participant outcomes including Beck Depression Inventory II, psychological well-being, health and social care utilisation, service satisfaction, substance misuse, record of priority/non-priority debts, life events and difficulties, and health economic outcomes. This study was terminated early during the pilot phase due to recruitment delays, and only randomised 32 participants in the intervention group (receiving debt advice from the Citizens Advice Bureau and shared biopsychosocial assessment in GPs) and 29 in the usual care group. There is only descriptive data of clinical and cost effectiveness outcomes. The data reported appear to suggest no clear differences between groups in the outcomes above. The evidence is clearly **uncertain** given its pilot status, early termination, and small sample size.

Schickedanz (2023) reports a pilot RCT of clinic-based financial coaching for low-income parents at the paediatric primary care clinic, USA(2). It randomised 81 parent-infant dyads (35 in the intervention group, and 46 in the control group) and evaluated the impact of clinic-based financial coaching on the adherence to recommended preventative care paediatric visits and vaccinations in the first six months of life. This study suggested that the rate of missed visits among those in clinic-based financial coaching (0.46 missed of four recommended visits) was half that of controls (1.07; mean difference = 0.61 visits missed; P = 0.01). Intervention participants were more likely to have up-to-date immunisations each visit (relative risk = 1.26; P = 0.01) with fewer missed vaccinations by the end of the 6-month preventative care visit period (2.52 vs 3.8 missed vaccinations; P = 0.002). The evidence however has some **uncertainties** due to the **methodological limitations** of this trial (small

sample size, likely unblinding of outcome assessors) and the **indirect relevance** of the US evidence to the UK context.

In a UK-based, quasi-experimental, controlled study, Woodhead (2018) evaluated the impacts and cost-consequences on mental health and service use by involving local Citizens Advice service staff at general practices to provide specialist welfare benefits and debts advice.(4) They included 278 intervention and 633 control participants and followed up them for three months. The study suggested that co-located welfare advice improved short-term mental health and well-being (β coefficient for well-being scores 1.29, 95% CI 0.25–2.32), reduced financial strain (ratio of odds ratios (rOR)=042, 95% CI 0.23–0.77), resulted in considerable financial returns (£15 financial gains per capita per £1 of funder investment), but did not impact on 3-month consultation rate. Common mental disorder scores were reduced among women (rOR = 0.37, 95% CI 0.20–0.70) and Black advice recipients (rOR=0.09, 95% CI 0.03–0.28). The evidence may have **limitations** as the study used quasi-experimental design, but propensity matching and adjusted analysis were used to control for confounding and missing rate (5%) is low. The intervention involved provision of welfare benefits advice, which appears broader than financial advice provision, thus the evidence may be **indirectly relevant**.

Egan and Robison (2019) evaluated the impacts of integrating money advice services into nine general practices, Scotland, on outcomes including advice referrals and engagement with services, financial gains. (7) This study suggested that this integration resulted in 654 advice referrals and around £1.5 million in financial gains for 182 people over 12 months in the nine practices evaluated. In comparison with this, over a comparable timescale, there are 1,264 referrals under the 'locality model' – a health centres-based model that allows health professionals to refer people to advice services in seven health centres across northeast Glasgow. This means that the nine practices alone resulted in the equivalent of more than half of all referrals across northeast Glasgow. The researchers' conservative estimate suggested that the return for every £1 invested into the integration was £25. This study however used historically controlled cohort design, thus having no contemporaneous comparison between groups. It is **unclear** whether integrating money advice services into GP is effective compared with alternative interventions.

3.3 People's experience of debt and financial advice and services (Q2)

We identified four UK-based primary studies with qualitative data for Q2. No evidence synthesis was identified.

Burrows (2011) reported a qualitative study in the UK that explored the views and experiences of service users (n = 12) and primary care and Citizens Advice Bureau advisor staff (n = 22) in using or providing Citizens Advice Bureau services located in primary care. (6) The confidential, non-stigmatising and familiar environment of a GP surgery; the ability to make appointments; and the availability and continuity of an experienced advisor were considered as key positive aspects of providing Citizens Advice services in GP. Perceived benefits of using the services include financial gain, managed debt, beneficial social and mental health impacts for patients.

Egan and Robison (2019) evaluated people's experience of integrating money advice services into general practices in Scotland. (7) Such integrations were viewed as creating a 'trusted hub', and some practice staff also reported an easing of workload and reduction in welfare-related appointments.(7)

In the pilot trial of Gabbay (2017), researchers also collected qualitative data from participants allocated to the intervention arm of using a primary care debt counselling advice service.(3) Participants' experiences were largely positive, and two aspects of Citizens Advice Bureau involvement were considered useful. First, the advisor role acting as a conduit between the participants and other agencies, to facilitate communication and negotiation about debt issues. Second, identifying possible sources of additional financial support and discretionary payments. A small number of participants viewed their appointment with the GP as psychologically supportive, providing validation for their concerns and evidence of GPs' interest in their current difficulties.

Reading (2002) collected qualitative data on access, use of the advice service, and the effect on the health of family members in the UK.(1) Researchers suggested that the support of health visitors impacts on uptake of the service. All the women who had used the service were positive about the content and value of the advice they received. Many of the women observed positive benefits on their mental health from the advice about financial and material problems.

3.4 Barriers and facilitators for delivery of and access to debt and financial advice and services (Q3)

We included one scoping review and a survey for Q3. Parry (2021) presents a scoping review that mapped screening tools used in primary care settings to identify patients' financial needs; described the key types of primary care-based interventions; and summarised barriers and facilitators to successful implementation.(5) Given its scoping nature, only evidence on barriers and facilitators is relevant and summarised in this RES. Parry and colleagues suggested that barriers to implementing primary care-based screening tools and interventions include not just cost and time, but also navigating three different things: the complexity of social welfare system, the difficulty of billing for non-clinical services, and both patients' and care providers' emotions about what can be stigmatising topics.(5) This review included largely US literature, and the tools and interventions included are largely related to welfare rights such as housing, food insecurity. Thus the evidence has **indirect relevance** for this RES.

Sharpe and Bostock (2002) reported a survey of with psychological therapists in Northumberland, UK, who supported people with debt and mental health problems.(8) The survey was not conducted in GPs or pharmacies, but possibly in community settings. The findings are largely barriers and facilitators from psychological therapists' perspective to accessing debt advice services of advice agencies such as Citizens' Advice Bureaux. Some therapists considered ***limited access to help*** as a barrier to debt advice use, suggesting that services were not necessarily accessible for all service users and home visits from advice agencies and health professionals' support in seeking help from

outside agencies would be useful for service users. Other barriers identified include: ***staff and service users' lack of awareness of debt advice services, difficulties of accessing Welfare Benefits, and limited support and information for health staff***.

4. References

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Produced by the NIHR Applied Research Collaboration Greater Manchester
12 October 2023.

The information in this report is correct at the time of printing.